

# UAE Paramedic Case Generator

Session Summary Report

Generated: 16/09/2026, 07:12:15

## Case Information

**Case ID:** y1-015  
**Case Title:** Severe Allergic Reaction - Restaurant  
**Category:** respiratory  
**Priority:** CRITICAL  
**Complexity:** BASIC  
**Year Level:** 1st-year  
**Patient:** 32yo female  
**Location:** Seafood restaurant, Al Maryah Island, Abu Dhabi

## Simulation Objective

**Objective:** Recognise life-threatening anaphylaxis, give intramuscular adrenaline without delay, and reassess airway, breathing, circulation and response after every intervention.

**Skills Focus:** Structured ABCDE assessment, IM adrenaline site and dose, High-flow oxygen and monitoring, Closed-loop reassessment and hospital pre-alert

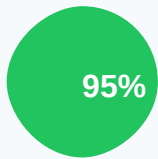
## Dispatch Information

Call Reason: 32-year-old female difficulty breathing after eating at restaurant, swollen face  
Time of Day: evening  
Caller Info: Restaurant manager (anxious)

## Initial Vital Signs

BP: 85/50 | Pulse: 125 bpm | RR: 28/min | SpO2: 93% | GCS: 15/15 | Glucose: 6.2 mmol/L

## Performance Summary



**Excellent**

125 / 125 points

Time: 08:42

Base score: 100%

**-5%** Inappropriate: Naloxone 0.4mg — Naloxone used without clear opioid physiology.

**Adjusted score: 95%**

## Performance Breakdown

### Overall: Excellent (95%)

Excellent performance. Systematic, thorough assessment with the decisive intervention given early.

#### Assessment & Recognition

96%

Systematic ABCDE; key findings identified.

#### Clinical Management

94%

Adrenaline-first, oxygen and fluids supported.

#### Timing & Efficiency

90%

Prompt first intervention.

#### Patient Safety

100%

No contraindicated or allergenic drugs given.

#### Strengths

- + Strong systematic assessment (ABCDE).
- + Interventions improved oxygenation.

#### Focus Next

- > Reassess after each intervention.

## Transport & Handover

|                     |                  |
|---------------------|------------------|
| Priority            | Lights & Sirens  |
| Position            | Sitting Upright  |
| Pre-alert           | Yes - Nearest ED |
| Working Dx          | Anaphylaxis      |
| Arrival on scene    | 00:18:05         |
| Departed (handover) | 00:26:47         |
| On-scene time       | 08:42            |
| Time to first Rx    | 1:12             |

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## Completed Actions

- + Scene Safety & BSI (Critical)
- + Assess airway patency (Critical)
- + Assess breathing rate (Critical)

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## Key Learning Points

- 1 Anaphylaxis = acute multi-system allergic reaction involving airway/breathing AND/OR circulation - skin signs alone are NOT anaphylaxis
- 2 IM adrenaline (0.5mg / 500mcg for adults) is the ONLY first-line life-saving treatment - give it IMMEDIATELY
- 3 Inject into the anterolateral thigh (vastus lateralis) - NOT the deltoid, NOT subcutaneously
- 4 Adrenaline works within minutes: vasoconstriction (raises BP), bronchodilation (relieves wheeze), reduces oedema
- 5 Can repeat IM adrenaline every 5 minutes if no improvement - there is no maximum dose in anaphylaxis
- 6 Antihistamines (chlorphenamine) treat itch and urticaria only - they do NOT treat airway or cardiovascular collapse
- 7 Positioning matters: flat + legs up for shock; sitting up if respiratory distress predominates
- 8 ALL anaphylaxis patients MUST go to hospital - risk of biphasic reaction (recurrence hours later)
- 9 Common triggers in UAE: shellfish, nuts, bee/wasp stings, medications (antibiotics, NSAIDs)
- 10 Always ask: has the patient used their EpiPen? Do they have one? (This patient left it at home)
- 11 Anaphylaxis vs allergic reaction: if any airway compromise, breathing difficulty, or hypotension -> it is anaphylaxis

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## Applied Treatments & Interventions

### 1. IM adrenaline matched to anaphylaxis

Applied at: 07:12

### 2. Oxygen matched to hypoxia

Applied at: 07:12

### 3. Bronchodilator matched to wheeze

Applied at: 07:12

#### 4. Naloxone used without clear opioid physiology

Applied at: 07:12

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## Management Summary

### Immediate

- Scene safety and BSI
- Remove allergen exposure (move patient away from food)
- Call for help early
- IM Adrenaline 0.5mg (500mcg) into anterolateral thigh - THIS IS THE PRIORITY
- High-flow oxygen 15L/min via non-rebreather mask
- Lay patient flat with legs elevated (if tolerated - may need to sit up if severe wheeze)
- Monitor airway continuously for deterioration

### Definitive

- Reassess after 5 minutes - repeat IM adrenaline if no improvement
- IV access and IV fluid bolus (500ml-1L normal saline)
- Transport to hospital - all anaphylaxis patients need observation (biphasic risk)
- Antihistamine (chlorphenamine 10mg IM) - adjunct only, NOT a substitute for adrenaline
- Hydrocortisone 200mg IV - prevents biphasic reaction (takes hours to work)
- Minimum 6-hour observation in hospital post-anaphylaxis

### Monitoring

- Continuous airway assessment - listen for stridor development
- SpO2 continuous monitoring
- Blood pressure every 2-3 minutes initially
- Heart rate and rhythm
- Respiratory rate and work of breathing
- Level of consciousness
- Rash progression or resolution
- Watch for biphasic reaction (can occur up to 72 hours later)

### Transport

- Pre-alert hospital: anaphylaxis, adrenaline given, vital signs
- Transport with ongoing monitoring
- Have second dose of adrenaline ready
- Position: flat with legs elevated OR sitting up if respiratory distress predominates
- Rapid transport - do not delay on scene

## Vital Signs Summary

|         | BP     | HR  | SpO2 | RR |
|---------|--------|-----|------|----|
| Initial | 85/50  | 125 | 93%  | 28 |
| Final   | 110/70 | 100 | 97%  | 18 |

Snapshots recorded: 2

## Common Pitfalls to Avoid

- (!) DELAYING adrenaline - this is the #1 error. Give adrenaline FIRST, not antihistamines
- (!) Giving antihistamines instead of adrenaline - antihistamines do NOT treat anaphylaxis
- (!) Wrong route: giving adrenaline IV instead of IM (IV adrenaline can cause fatal arrhythmias in non-arrest patients)
- (!) Wrong site: injecting into deltoid or buttock instead of anterolateral thigh
- (!) Not repeating adrenaline if first dose does not work within 5 minutes
- (!) Standing the patient up - this can cause cardiac arrest in anaphylaxis (empty ventricle syndrome)
- (!) Not monitoring the airway - angioedema can progress to complete airway obstruction
- (!) Forgetting to remove the allergen (move patient away from the prawns)
- (!) Not transporting to hospital - biphasic reactions can be fatal
- (!) Confusing anaphylaxis with panic attack - always check for skin signs, wheeze, and hypotension

## Instructor Assessment Feedback

[IMPORTANT]

Phase: primary-survey

07:12

**You recognised the multi-system allergic reaction and correctly prioritised intramuscular adrenaline, oxygenation and repeat airway assessment.**

Missed: The post-treatment reassessment did not explicitly document whether the facial swelling, work of breathing and blood pressure had improved before packaging.

Action: After every treatment, repeat and document a focused ABC assessment, including the airway, respiratory effort, oxygen saturation, pulse and blood pressure.

Summary: 0 critical | 0 areas for improvement | 0 strengths noted

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## Further Study Resources

### Guidelines

|                              |                                                                 |   |
|------------------------------|-----------------------------------------------------------------|---|
| [World Allergy Organization] | <a href="#">World Allergy Organization Anaphylaxis Guidance</a> | * |
| [Resuscitation Council UK]   | <a href="#">Resuscitation Council UK: Anaphylaxis Algorithm</a> | * |

### Articles

|            |                                               |   |
|------------|-----------------------------------------------|---|
| [REBEL EM] | <a href="#">Anaphylaxis - Evidence Review</a> | * |
|------------|-----------------------------------------------|---|

Resources from: NICE, Resuscitation Council UK, Radiopaedia, EMDocs, REBEL EM, ALiEM, EM Cases, EMCrit.